

HOW TO OPEN A GROUP HOME BUSINESS in Wisconsin

Adult family homes & CBRFs under Family Care and IRIS — serving adults with disabilities.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Wisconsin model — read this first

“Group home” in Wisconsin usually means a licensed residential setting where a few adults with disabilities live and receive care above the level of room and board. The state funds this through its long-term care programs, run by the Department of Health Services (DHS), and it does something most states don't: it delivers that care through MANAGED CARE and SELF-DIRECTION. Two things make Wisconsin distinctive, and they run through everything below. First, the money doesn't come from plain fee-for-service Medicaid — it comes through Family Care (where the state pays a managed care organization, or MCO, that buys services for its members) or through IRIS (Include, Respect, I Self-Direct — where the participant manages their own budget and hires their own providers). Second, Wisconsin treats the two home sizes very differently: a 1–2 bed adult family home is CERTIFIED, while a 3–4 bed adult family home is LICENSED by DHS. A larger setting (5 or more) is a Community-Based Residential Facility, or CBRF — a bigger, more heavily regulated operation.

The two ways in. Path A — a CERTIFIED 1–2 bed adult family home (AFH): you live in the home, and one or two adults live with you and receive care. Certified through your MCO (Family Care) or DHS (IRIS), not licensed. Smallest, fastest, closest to a host home. Path B — a LICENSED 3–4 bed adult family home: three or four unrelated adults live in a residence you operate, licensed by the DHS Division of Quality Assurance (DQA) under Wis. Admin. Code ch. DHS 88, with a licensing survey. A bigger operation with more oversight — the true “group home.” This guide walks both, and is honest about which is realistic when. (A CBRF of 5+ beds is a further step up; we flag it where it matters.)

The names you'll deal with. DHS = Department of Health Services (runs long-term care). DQA = its Division of Quality Assurance (licenses/certifies homes). Division of Medicaid Services = runs IRIS. MCO = the managed care organization that runs Family Care in your area and contracts with and PAYS providers (Inclusa, Lakeland Care, My Choice Wisconsin, and others). IRIS = the self-directed program, where participants use a budget and a fiscal employer agent (FEA) to pay providers, with an IRIS consultant to help. ADRC = the local Aging and Disability Resource Center — the front door for eligibility and enrollment. Long-Term Care Functional Screen = the assessment that determines functional eligibility.

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened a home — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or billing advice; requirements, rates, and codes change, so verify each step with DHS, DQA, your ADRC, the MCOs, IRIS, your municipality, and qualified local counsel (see Part 12) before you rely on it.

The journey at a glance

These milestones take you from idea to your first payment — whether you start with a certified 1–2 bed home or a licensed 3–4 bed home.

1

Decide your path & set up the

Path A (certified 1–2 bed) or Path B

	business	(licensed 3–4 bed); entity, EIN, NPI, insurance (Parts 3–4)
2	Confirm there's a need where you live	Call your ADRC and local MCO(s)/IRIS before you invest — contracts aren't offered everywhere (Part 4)
3	Get certified or licensed	Certify a 1–2 bed via your MCO/IRIS, or apply for a 3–4 bed DHS 88 license & pass the survey (Part 5)
4	Pass background checks & training	Caregiver checks and 15 hours of required training including fire safety and first aid (Part 5)
5	Contract with your funding source	Contract with the Family Care MCO(s) in your area, and/or serve IRIS participants (Part 5)
6	Ready the home	AFH vs. CBRF settings, HCBS rights, water temperature, and fire safety (Part 6)
7	Staff & write your policies	Caregivers/DSPs, RN oversight, documentation the state reviews (Parts 7–8)
8	Get a person placed	Functional screen, Family Care/IRIS enrollment, care plan, and the MCO/IRIS match (Part 9)
9	Deliver care & get paid	Bill your MCO or the IRIS FEA against the authorized plan (Part 10)

Rule of thumb. Start smaller than you think, and confirm the need FIRST. Wisconsin's managed-care contracts aren't available in every municipality, so before you buy a house or file a license, one call to your ADRC and the local MCO(s) or IRIS can save you months. Many providers begin with a certified 1–2 bed home — the family model — learn the system, and grow into a licensed 3–4 bed home or a CBRF once they know the terrain.

Before you begin — a realistic picture

Two honest truths will save you time and money. First, this is a relationships-and-documentation business funded by Medicaid managed care and self-direction, not a real-estate play or a normal small business: your revenue depends on getting certified or licensed AND contracting with an MCO or serving IRIS participants, and then on the MCO or the participant choosing your home — so your reputation with your ADRC, the MCO care managers, and IRIS consultants is your most valuable asset. Second, the money arrives on a chain and on a delay: certification or the DHS 88 license takes time, MCO contracting takes more time on top of that, and the underlying Medicaid and long-term care enrollment for a PERSON can itself take up to three months — and you don't bill until someone is placed and their services are authorized. So you need patience and a working-capital cushion to carry any staff and setup costs through the ramp. Start with a certified 1–2 bed home if you can, learn the system, and grow into a licensed home once you know the terrain.

Here's how the rest of this guide is organized: Parts 1–2 explain what the business is and who regulates and pays you; Part 3 sets up the business itself, with the EIN, NPI, bank account, and insurance broken all the way down; Part 4 walks the two paths in; Part 5 is the certification-or-license plus contracting chain; Part 6 covers the home, water safety, and fire safety; Parts 7–8 are staffing and the policies and records the state reviews; Part 9 is how a person actually gets placed with you through Family Care or IRIS; Part 10 is how you get paid; Part 11 covers costs, timeline, and getting placements; and Part 12 collects every phone number and script in one place, with Milwaukee and Madison worked out. Read it once end to end, then keep it beside you as a checklist.

PART 1

What This Business Is

In short: *Providing residential care to adults with disabilities in a small home — a certified 1–2 bed adult family home, a licensed 3–4 bed adult family home, or (larger) a CBRF. Paid through Family Care (an MCO) or IRIS (self-direction), after you're certified or licensed and contracted.*

A “group home” business in Wisconsin is really a residential long-term-care business. You help an adult with a disability — often an intellectual or developmental disability, a physical disability, or a mental-health need — live successfully in the community with daily support, supervision, help with health and medications, and a real home life, rather than in an institution. Wisconsin's residential options run along a spectrum of size: a 1–2 bed adult family home (the family model, where the provider lives in the home), a 3–4 bed adult family home (the classic small group home), and a Community-Based Residential Facility (CBRF) of 5 or more residents (a larger, more regulated setting). The bigger the setting, the heavier the oversight and the more it looks like a facility rather than a home.

Be clear-eyed about what kind of business this is. This is a human-services business built on relationships and documentation, funded by Medicaid through managed care and self-direction — not a real-estate play. Your value is the quality and reliability of the care you provide and your ability to keep it compliant and well-documented, because that's what DHS certifies or licenses, what the MCO or IRIS authorizes and pays for, and what keeps people safe. Go in understanding that the care and the paperwork ARE the business.

Why this business — and why now

Demand for community-based residential care for people with disabilities is high and durable: Wisconsin, like every state, has moved people out of institutions and into the community, families overwhelmingly want a real home for their loved one, and Family Care and IRIS fund exactly this. Because the MCO or the participant chooses the home, a reliable, well-run provider that builds relationships with care managers and IRIS consultants can keep its beds full. And you can start lean — a certified 1–2 bed home where you live with one or two adults — and grow into a licensed 3–4 bed home or a CBRF as you learn the system.

Be honest about the two realities that trip up new providers. (1) *Managed care and self-direction add steps: you don't just get a state license and bill Medicaid — you get certified or licensed AND contract with each MCO (or serve IRIS participants) before a dollar flows, and contracts aren't available in every municipality.* (2) *Room and board is separate from your pay: the funding pays for the CARE and services above the level of room and board — the resident pays their own rent and food from their own income (often SSI). Plan your business — and explain it to families — around both.*

PART 2

Who Regulates You & Who Pays You

In short: *DHS (through DQA) certifies or licenses your home; the Family Care MCO or IRIS authorizes and pays for the care. The ADRC is the front door where each person's eligibility and enrollment begin.*

DHS / DQA — the Division of Quality Assurance (your certifier/licenser)

DHS runs Wisconsin's long-term care system. Its Division of Quality Assurance (DQA) certifies 1-2 bed adult family homes and licenses 3-4 bed adult family homes (under Wis. Admin. Code ch. DHS 88) and CBRFs (under DHS 83). Without the right certification or license you cannot legally operate the home. For a 3-4 bed AFH you apply through the DQA Provider Portal using a MyWisconsin ID, and you pass an initial licensing survey.

The MCOs — Family Care managed care (your payer, one way)

Under Family Care, the state pays a managed care organization (MCO) a monthly amount and the MCO buys services for its members. The MCOs — such as Includa, Lakeland Care, and My Choice Wisconsin, varying by county — contract with providers and pay their claims. For a certified 1-2 bed home, the MCO itself certifies you; for a licensed 3-4 bed home, you hold the DHS 88 license and then contract with the MCO. Either way, an MCO care manager helps build each member's plan and, with the member's choice, matches them to your home.

IRIS — self-direction (your payer, the other way)

Under IRIS (Include, Respect, I Self-Direct), run by the DHS Division of Medicaid Services, the participant manages their own budget and hires their own providers, with an IRIS consultant to help and a fiscal employer agent (FEA) that actually pays providers from the budget. To serve IRIS participants in a 1-2 bed home you certify through the DHS AFH Provider Portal (email DHSIRISAFH@dhs.wisconsin.gov to start); the participant then chooses your home and pays you through the FEA.

The ADRC — the front door

Every person starts at their local Aging and Disability Resource Center (ADRC), which checks eligibility with the Long-Term Care Functional Screen and helps the person choose Family Care or IRIS. You'll deal with the ADRC constantly — it's where placements begin. There's an ADRC in all 72 counties; the statewide line is 844-WIS-ADRC (844-947-2372).

What this means for you: the path to payment is a chain — get certified or licensed by DHS/DQA, then contract with an MCO (or certify to serve IRIS participants), and finally get matched to a person whose plan is authorized. Each link takes time. Start them in the right order and treat the ADRC, MCO care managers, and IRIS consultants as key relationships, because they and their members choose the home. A home that's certified/licensed, contracted, and known to the local care managers is a home that fills.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Wisconsin you form the LLC through the Department of Financial Institutions (DFI). Your entity, EIN, and NPI all have to be in place and consistent before you certify or apply for a license and contract with an MCO, because they're checked along the way. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization with the Wisconsin DFI.** Online at the DFI website — the LLC filing fee is \$130 online (about \$170 by paper). Name the LLC and list a registered agent (a person or service with a Wisconsin street address — this can be you).
- 2. Adopt an Operating Agreement.** The internal document that sets ownership and management — your bank, your certification/license file, and your MCO contracts will reference your structure.
- 3. File your annual report every year.** Wisconsin LLCs file an annual report with DFI to stay in good standing — the fee is \$25 for a Wisconsin LLC. Calendar it so you never lapse.
- 4. Confirm local business licenses and — critically — local ordinances.** Check what city, village, or town business licenses apply, and connect with your municipality about ordinances that govern the home (DHS 88 specifically requires this for a 3–4 bed AFH).

No newspaper publication in Wisconsin. Wisconsin has NO LLC publication requirement — forming the entity is a straightforward DFI filing (\$130 online) plus the \$25 annual report each year. That's simpler and cheaper than the publication states.

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll as a Wisconsin Medicaid provider and to bill your MCO (Family Care) or the fiscal employer agent (IRIS) for care. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.

2. Start a Type 2 (Organizational) application. Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits an adult-family-home / residential developmental-disabilities provider — confirm the exact one your MCO and IRIS want. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Wisconsin branches — Associated Bank, BMO, U.S. Bank, Nicolet National Bank, or a local Wisconsin credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance & bonding — what you need, who sells it, and what to ask

A residential-care provider brings staff into a home and supports vulnerable adults, so you need coverage built for a human-services / developmental-disabilities provider — and DHS, the MCOs, and IRIS will expect proof:

Coverage	What it protects against	Must-have?
General & Professional Liability	Injury or property damage, plus claims about the CARE and supervision you provide	Yes — foundational
Abuse & Molestation	Allegations of abuse of a resident — general liability EXCLUDES this	Yes — high exposure
Directors & Officers / Management	Regulatory, employment, and governance claims	Recommended for an agency
Non-Owned & Hired Auto	Caregivers driving their own cars to support or transport residents	Yes — critical
Workers' Compensation	Caregiver injuries	Required in WI (3+ employees)
Property	Coverage on the home you own or lease	If you own/lease the home

Abuse & molestation is your highest exposure. Supporting vulnerable adults means abuse & molestation coverage — EXCLUDED by standard general liability — is essential, with a real limit, not a token sub-limit. Confirm it explicitly.

Real places to get it

Buy through an independent agent or broker who writes HUMAN-SERVICES / developmental-disabilities and residential-care provider programs — this is a specialty niche. Carriers and programs active in DD/IDD and residential care include NSM/Irwin Siegel Agency (a social-services specialist), Negley Associates, Philadelphia Insurance (PHLY), and CNA. Find a local Wisconsin agent and ask specifically for an adult-family-home / CBRF / developmental-disabilities provider program. Confirm the carrier writes this in Wisconsin before you rely on a quote.

What to ask the insurance agent

“Do you write adult family homes / CBRFs / developmental-disabilities providers in Wisconsin — which carriers, and what's the minimum premium?”

“Is abuse & molestation included, at what limit, and is it a sub-limit or a full limit?”

“Do you include non-owned & hired auto for my caregivers driving the residents we serve?”

“What limits do DHS, the MCOs, and IRIS expect?”

“Can you set up Wisconsin workers' comp for my caregivers?”

On bonding: Wisconsin providers who hold or manage resident funds are expected to protect that money — confirm with DHS/DQA and your MCO whether a surety bond or a separately maintained resident-funds account is required for your setting, and keep any resident money strictly separate from the business's own funds.

3.6 Keep your paperwork consistent — every step downstream checks it

One practical warning that saves Wisconsin providers real time: your legal name, ownership, EIN, and NPI have to line up EXACTLY across every step that follows — your certification or DHS 88 license application, your Wisconsin Medicaid enrollment, and each MCO contract (or IRIS certification). Managed care means your information passes through several organizations before a claim pays, and a mismatch at any handoff — a slightly different business name here, an old address there — is one of the most common and most frustrating causes of a stalled enrollment or a rejected claim. Set the entity up once, correctly, and make every downstream document point at the same clean legal picture. It's worth an hour with your accountant or attorney up front to confirm the structure, because untangling a mismatch after you've applied costs far more than getting it consistent now. And keep your certification/license, enrollment, and MCO contracts calendared for renewal — letting any one of them lapse (or missing the DFI annual report) stops your payments even when the care is going perfectly.

The business foundation to have in hand before you certify or apply for a license:

- Your formed Wisconsin LLC (Articles of Organization filed with DFI, annual report current) and Operating Agreement
- Your EIN confirmation (CP-575) from the IRS
- Your Type 2 (organizational) NPI from NPPES
- A business bank account opened in the entity's exact legal name
- Bound insurance — general/professional liability, abuse & molestation, non-owned auto, and workers' comp
- A single, consistent legal name, ownership list, and address used identically on every document

With those six things in place and consistent, you're ready to build the certification or license application in Part 5 on a clean foundation — and you've removed the paperwork mismatches that stall more Wisconsin applications than any substantive problem does.

PART 4

The Two Paths — Certified 1-2 Bed (A) or Licensed 3-4 Bed (B)

In short: Path A is the fast, small way in — a certified 1-2 bed adult family home where you live with one or two adults. Path B is the real group home — a licensed 3-4 bed adult family home under DHS 88. Pick the one that fits where you are today; many start with A and grow into B (or a CBRF).

Before EITHER path — confirm the need. This is the single most important call you'll make. Contact your ADRC and the local MCO(s) or IRIS and ask whether services are actually needed in YOUR municipality before you invest — contracts for certain adult family homes aren't offered everywhere. This one call can save you months and a lot of money. For a 3-4 bed home, also connect with your municipality about local ordinances (required by DHS 88.04(2)(a)).

4.1 Path A — a certified 1-2 bed adult family home

A 1-2 bed AFH means YOU live in the home and one or two adults live with you and receive care above the level of room and board. It's the fastest way in and the true family model — closest to a host home. A 1-2 bed AFH is CERTIFIED (not licensed): for Family Care the MCO certifies you; for IRIS you certify through the DHS AFH Provider Portal.

- 1. Make sure it fits your life.** Everyone in the household is part of this, so it's a family decision — go in with eyes open about the good days and the hard ones.
- 2. Confirm there's a need where you live.** Call your ADRC and the local MCO(s) or IRIS to confirm services are needed in your municipality before you go further.
- 3. Get certified.** For Family Care, the MCO certifies you; for IRIS, certify through the DHS AFH Provider Portal (email DHSIRISAFH@dhs.wisconsin.gov to start). Certification follows the Wisconsin Medicaid Standards for Certified 1-2 Bed Adult Family Homes (P-00638).
- 4. Pass background checks & training.** Complete caregiver background checks (including the DHS Caregiver Registry) for everyone covered by the rule, and 15 hours of training within your first 90 days (the WisCaregiver Careers program meets this) — medications, fire safety, first aid, resident rights, and abuse prevention.
- 5. Get your home ready.** Meet the home-environment and HCBS-settings standards — privacy and dignity, safe water temperature, well-water testing if you're not on public water, and the safety basics (see Part 6).
- 6. Get matched, and get paid.** The MCO (or the IRIS participant) refers people whose plan calls for a 1-2 bed AFH, and pays you per your contract or the IRIS budget.

The difficulty-of-care tax note. Because you LIVE in the home in a 1-2 bed AFH, your payments may qualify for the IRS "difficulty-of-care" exclusion (potentially non-taxable). Confirm with a tax professional — it can meaningfully change the economics of the family-model arrangement.

4.2 Path B — a licensed 3-4 bed adult family home

Path B is more work, more capacity, and more control: three or four unrelated adults live in a residence you operate, and DHS/DQA licenses you under Wis. Admin. Code ch. DHS 88 with an initial licensing survey. It's the classic group home — a real, scalable business — and Part 5 walks the license path step by step. Most operators who go this route either have direct experience in disability services or hire someone who does, because your program statement and application have to show DQA you understand the standards and can deliver.

Beyond 3-4 beds sits the CBRF — a Community-Based Residential Facility of 5 or more residents, licensed under DHS 83. A CBRF is a bigger, more heavily regulated setting (with its own staffing, training, fire, and building requirements) and a larger capital and compliance commitment. Most new providers grow into a CBRF only after running an adult family home; if you're considering one, treat it as a separate, larger project and confirm the DHS 83 requirements with DQA before you commit.

PART 5

Get Certified or Licensed & Contract with Your Funding Source

In short: For Path A, get certified through your MCO or IRIS. For Path B, apply for the DHS 88 license and pass the survey. Either way, background checks and 15 hours of training are required — and a license or certification lets you operate, but the MCO contract or IRIS participants are how you actually get paid.

5.1 Path A — get certified (1–2 bed)

- 1. Confirm the need, then start certification.** With your ADRC and MCO/IRIS having confirmed a need, begin certification: the MCO certifies you for Family Care, or you certify through the DHS AFH Provider Portal for IRIS (DHSIRISAFH@dhs.wisconsin.gov).
- 2. Meet the P-00638 standards.** Certification follows the Wisconsin Medicaid Standards for Certified 1–2 Bed Adult Family Homes (P-00638) — the home-environment, HCBS-settings, and care standards you'll be held to.
- 3. Background checks & training.** Complete caregiver background checks (including the DHS Caregiver Registry) and 15 hours of training within 90 days.
- 4. Contract / get authorized.** For Family Care you're certified and paid by the MCO; for IRIS the participant authorizes and pays you from their budget through the FEA.

5.2 Path B — get licensed (3–4 bed, DHS 88)

- 1. Set up the business & confirm the need.** Form your entity with DFI and get your EIN (Part 3), confirm with an MCO or IRIS that services are needed in your municipality, and connect with your municipality about ordinances (required by DHS 88.04(2)(a)).
- 2. Apply through the DQA Provider Portal.** A 3–4 bed AFH is licensed by DHS/DQA under DHS 88. Apply through the DQA Provider Portal using your MyWisconsin ID, complete an applicant compliance statement, and upload your program statement.
- 3. Pass the initial licensing survey.** DQA conducts an initial licensing survey of the home before you can operate. There's a licensing fee of \$171 every two years.
- 4. Background checks & training.** Complete caregiver background checks (Wis. Stat. § 50.065 and DHS 12) for all caregivers, and ensure the licensee and each service provider completes 15 hours of approved training — including fire safety and first aid — before or within six months of starting to provide care (resident-rights training at onboarding and repeated yearly).
- 5. Contract with your funding source.** A license lets you operate; an MCO contract (or IRIS participants) is how you get paid. Contract with the Family Care MCO(s) in your area, and/or serve IRIS participants who choose your home.
- 6. Put policies and staffing in place, then serve.** Put your policies and procedures in place (PolicyReady manuals), hire and train your caregivers, and make sure each resident has the required person-centered plan and rights. Then accept referrals, deliver care, and bill your funding source (Parts 9–10).

What this means for you: licensing and certification are quality gates, not forms. DQA and the MCOs want evidence — real policies, a real program statement, real capacity — that you can safely support people. The providers who get through cleanly treat the standards as the blueprint for the whole operation, so building your policies and staffing to the standard does double duty: it earns your certification or license AND it's how you'll actually run.

PART 6

The Home — Settings, HCBS Rights & Fire Safety

In short: A 1–2 bed or 3–4 bed adult family home is a private residence held to the home-environment, HCBS-settings, and (for 3–4 bed) DHS 88 standards — not a commercial facility. A CBRF carries heavier building and fire requirements. In every case Wisconsin requires you to check local ordinances, and the home must genuinely be the person's home.

6.1 Certified vs. licensed — the key distinction

A 1–2 bed AFH is certified to the Wisconsin Medicaid Standards for Certified 1–2 Bed Adult Family Homes (P-00638) — a home you live in, held to home-environment and HCBS standards. A 3–4 bed AFH is licensed under DHS 88, with a licensing survey and a program statement. A CBRF (5+ residents) is licensed under DHS 83 with substantially more building, staffing, and fire regulation. And in every case, Wisconsin requires you to check with your local municipality about ordinances that govern the home and its operation (DHS 88.04(2)(a) for a 3–4 bed AFH).

6.2 “Do I need fire sprinklers?” — the honest answer

Adult family homes (both 1–2 bed and 3–4 bed) are small private residences, so a full commercial fire-sprinkler system generally is NOT required. What you MUST have is genuine fire safety — working smoke and carbon-monoxide detectors, fire extinguishers, clear exits, and a practiced emergency plan — and training in fire safety is part of the required 15 hours. A CBRF, by contrast, is a larger licensed facility whose fire and building requirements are heavier and may include sprinklers depending on size and occupancy. Because actual fire and building requirements come from Wis. Admin. Code ch. DHS 88 (or DHS 83 for a CBRF) and your local municipality — Wisconsin specifically requires you to check local ordinances — and can differ for ambulatory vs. non-ambulatory residents, confirm your specific home with your local municipality and fire authority BEFORE you buy or renovate.

6.3 The home checklist

- ☐ Certification (1–2 bed) or a passed licensing survey (3–4 bed) before anyone moves in.
- ☐ Working smoke and carbon-monoxide detectors, fire extinguishers, clear exits, and a written, practiced emergency plan.
- ☐ Safe water temperature (Wisconsin sets AFH water-temperature standards to prevent scalding) and, on a private well, tested well-water samples.
- ☐ HCBS settings rights: each resident's privacy, dignity, and respect; a person-centered plan; and freedom of choice about their setting and daily life.
- ☐ Local municipal compliance — confirm zoning and any local ordinances with your city, village, or town (required by DHS 88.04(2)(a)).
- ☐ Setting size respected — 1–2 or 3–4 for an AFH; a home of 5+ is a CBRF under DHS 83, a different license entirely.

Two things people get wrong. (1) *Certified vs. licensed changes your whole process — don't apply for a 3–4 bed license if you're really opening a 1–2 bed home, which is certified through your MCO or IRIS, not licensed by DQA.* (2) *Room and board is separate from your pay — AFH care is “above the level of room and board,” so the resident pays their own room and board from their income and your payment is for the care and services. Keep the two completely separate in your books and your agreements.*

PART 7

Who You Actually Need — Staffing

In short: In a 1–2 bed home (Path A), you and your household are the care provider, with substitute coverage for time off. In a 3–4 bed home (Path B) or a CBRF, plan for a licensee/operator, caregivers, substitute/respite caregivers, nurse oversight, and billing — one person can wear several hats at the start.

7.1 The roles

For a 1–2 bed AFH, you (and your household) are the care provider, living in the home. You'll arrange substitute/respite coverage so you can take time off, and complete your own training. You generally don't build a staff. For a 3–4 bed AFH or a CBRF, plan for these roles:

- **Licensee / operator:** runs the home, owns the license and compliance, and holds the MCO contracts.
- **Caregivers / service providers:** the trained staff who deliver care — each background-checked, with 15 hours of training including fire safety and first aid.
- **Substitute / respite caregivers:** to cover time off — also screened and trained.
- **Registered Nurse:** an AFH may provide up to seven hours per week of nursing care per resident — often part-time or contracted at first.
- **Administrative / billing support:** handles MCO and IRIS billing and documentation.

7.2 Recruiting and keeping caregivers — the real make-or-break

Direct-care staff are the heart of this business and the hardest thing to keep. Caregiver turnover is high across the whole field, and a home that can't reliably staff its shifts can't safely support people or keep its authorizations — so treat recruiting and retention as your core operation, not an afterthought. (This applies mainly to a 3–4 bed home or a CBRF; in a 1–2 bed home, your reliable substitute/respite coverage is the equivalent.)

- **Recruit continuously and hire for heart:** specific tasks can be trained; patience, reliability, and genuine care for people with disabilities cannot. Check references for dependability. The WisCaregiver Careers program is a recruiting and training resource.
- **Onboard to the standard:** real orientation, the required background and Caregiver Registry checks and the 15 hours of training, and a competency check before a caregiver supports someone alone — this protects residents and satisfies DHS.
- **Retain deliberately:** consistent schedules, correct and on-time pay, real support when a caregiver hits a hard day, and recognition. Every caregiver who quits costs you re-hiring, re-training, and often continuity for the resident they supported.

PART 8

Policies, Records & Staying Compliant

In short: *Your policies and documentation are what DHS certifies or licenses and what the MCO or IRIS pays against. Build them to the standard from day one — medications, emergencies, incident reporting, and the rights of the people you support.*

You keep clear documentation covering medications, emergencies, incident reporting, and the rights of the person you support — and the person-centered plan (the member-centered plan in Family Care, or the ISSP in IRIS) drives what you deliver and record. This documentation is both the certification/licensing requirement and the backbone of a safe, well-run operation. Build a simple system that keeps each person's plan, service records, and incident documentation current, because DQA reviews it at survey and the MCO can audit it. PolicyReady's Wisconsin manuals are built for exactly this — ready to customize, just add your details.

8.1 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 9

The Person's Journey — Functional Screen to Move-In

In short: *You can't get paid until someone is placed with you and their care is authorized through Family Care or IRIS. The path runs from the Long-Term Care Functional Screen at the ADRC, through enrollment in an MCO or IRIS, to the care plan and the match to your home.*

- 1. The person qualifies.** They're a Wisconsin adult, enrolled in Medicaid, and found functionally eligible through the Long-Term Care Functional Screen (done by the ADRC). This underlying enrollment can itself take up to three months.
- 2. They enroll in Family Care or IRIS.** At the ADRC, they choose their program and — for Family Care — their MCO, or — for IRIS — an IRIS consultant agency and a fiscal employer agent.
- 3. The plan calls for an AFH.** Their person-centered plan (the member-centered plan in Family Care, or the ISSP in IRIS) identifies a 1–2 bed or 3–4 bed adult family home (or a CBRF) as the right setting.
- 4. The match.** The MCO care manager (or the IRIS participant) chooses your home based on fit, your certification or license, and your available capacity.
- 5. Move-in.** With the plan and your contract in place, the person moves in and you're delivering a billable service.

What this means for you: because the member (or their care manager) chooses the home, your census depends on relationships and reputation, not marketing. Get known to your ADRC, to the MCO care managers, and to IRIS consultants in your area as the provider who supports people well, communicates, and handles a placement smoothly — and the matches follow.

Plan your business around the enrollment reality. *The underlying Medicaid and long-term care enrollment for a person can take up to three months, and contracts aren't available in every municipality — which shapes how you should plan. Don't build a model that assumes you'll fill every bed the month you're licensed; assume a gradual build as care managers learn to trust you and matches come through. Two practical moves help: start with a certified 1–2 bed home so you have revenue while you build, and stay in regular, helpful contact with the ADRC, the MCO care managers, and IRIS consultants in your area so that when one of their members is choosing a home, you're the name they think of first. Patience plus reliability is the whole strategy — the demand is real, but it arrives one authorized person at a time.*

PART 10

How You Get Paid

In short: Under Family Care the MCO pays you on your provider contract at a negotiated rate; under IRIS the participant pays you from their self-directed budget through the fiscal employer agent. Your payment is for the care, above room and board — which the resident pays separately.

10.1 Who pays you and how the rate is set

- **Family Care (the MCO):** the MCO pays you under your provider contract. Your rate is NEGOTIATED with the MCO — there's no single statewide fee schedule for AFH care, so this is a conversation with each MCO you contract with.
- **IRIS (the participant):** the participant pays you from their self-directed budget, through the fiscal employer agent (FEA). The rate is agreed within the participant's budget.
- **What the rate covers:** your payment is for the care and services above room and board. The resident pays their own room and board from their income (often SSI).
- **Where to confirm:** your MCO contract (Family Care) or the IRIS budget authorization is your source of truth for what you'll be paid — confirm it in writing before you serve.

10.2 The money math — how an adult family home pencils out

AFH economics are different from an hourly private-pay business — your revenue is the negotiated MCO rate (or the IRIS-budget rate) for the authorized care, and your biggest cost is caregiver labor. Lay it out with your own numbers:

- **Revenue:** the authorized care in each resident's plan, paid by their MCO at your contracted rate or from the IRIS budget through the FEA — for the CARE only, above room and board.
- **Cost:** caregiver wages and payroll taxes are the bulk (in a 3–4 bed home or CBRF); then RN oversight, administration, insurance, and the home itself. In a 1–2 bed home where you live, your own time is the main input, and the difficulty-of-care tax exclusion may apply.
- **The lever:** because rates are negotiated and set within the plan or budget, your margin comes from staffing efficiently to each resident's authorized plan and keeping your beds filled with authorized residents — utilization, not rate, is the number you manage.
- **Room and board stays separate:** the resident's rent and food come from their own income (usually SSI), NOT the MCO or IRIS care payment — keep that money and those agreements entirely separate from your service billing.

10.3 Your first payment

1. **Confirm your MCO contract (or the IRIS budget authorization) and the agreed rate.**
2. **Deliver care and keep your documentation.**
3. **Bill your funding source.** The MCO (Family Care) or through the FEA (IRIS).
4. **Get paid — and keep everything current.** Keep your certification/license, enrollment, and authorizations current so payment never lapses.

PART 11

What It Costs, How Long It Takes & Getting Placements

In short: A certified 1–2 bed home can move relatively quickly; a licensed 3–4 bed home takes longer through business setup, the DHS 88 application and survey, and MCO contracting. Placements come through the MCO or IRIS, so your relationships with care managers are your census.

11.1 Timeline & startup costs

A 1–2 bed AFH (Path A) can move relatively quickly once you've confirmed a need and started certification — background checks, training, and the certification review. A 3–4 bed AFH (Path B) takes longer: business setup, the DHS/DQA license application and initial survey, and contracting with an MCO. Remember the underlying Medicaid and long-term care enrollment for a PERSON can itself take up to three months. A 1–2 bed home's costs are modest — mainly your time, background checks, training, and getting your home ready. A 3–4 bed license adds business formation (\$130 DFI filing plus the \$25 annual report), insurance, the \$171 licensing fee every two years, staff payroll, and your policies and procedures. A CBRF is a much larger capital and compliance commitment. Confirm current fees and timelines with DHS/DQA and your MCO.

11.2 Getting your first placements

Placements don't come from advertising — they come through Family Care and IRIS and your reputation. Here's a simple plan:

- 1. Get known to your ADRC and confirm the need.** Call 844-WIS-ADRC (844-947-2372) to reach your county ADRC and confirm there's a need for your setting in your municipality.
- 2. Build relationships with MCO care managers and IRIS consultants.** They (and their members) choose the home. Be the provider they trust to handle a placement well, communicate, and never leave a shift unstaffed.
- 3. Be responsive and reliable.** Answer the phone, say yes to a good match, handle move-in smoothly, and follow through — one good placement earns the next referral.

PART 12

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your ADRC, your MCO/IRIS, and (for a 3–4 bed) DHS/DQA are the center of gravity, with your municipality close behind. Statewide contacts are the same for everyone; your municipal and fire offices are local. Here's the list and the scripts.

12.1 Statewide (same for everyone)

For...	Contact
Find your local ADRC	844-WIS-ADRC (844-947-2372) · dhs.wisconsin.gov/adrc
Licensing a 3–4 bed AFH	DHS Division of Quality Assurance — DQA Provider Portal (MyWisconsin ID)
Certifying a 1–2 bed AFH (IRIS)	DHS — DHSIRISAFH@dhs.wisconsin.gov (AFH Provider Portal)
IRIS program questions	DHS IRIS — 888-515-4747 · DHSIRIS@wisconsin.gov
Family Care / member services	800-362-3002 · Family Care GSR map for MCOs by county
Business, EIN & NPI	wdfi.org (DFI) · irs.gov (EIN) · nppes.cms.hhs.gov (NPI)

12.2 The local calls — and exactly what to ask

① Your ADRC and local MCO/IRIS — your most important calls

"Is there a need for a 1–2 bed / 3–4 bed adult family home in my municipality, and which MCOs contract here?"

"What's your process to certify or contract with a new adult family home, and how are rates set?"

② DHS/DQA (for a 3–4 bed) — licensing

"How do I start a 3–4 bed AFH license in the DQA Provider Portal, and what's on the initial licensing survey?"

③ Your municipality (city / village / town) — required

"I want to open an adult family home for [1–2 / 3–4] adults with disabilities at this address. What local ordinances or zoning apply? (Wisconsin requires you to check this — DHS 88.04(2)(a).)"

12.3 Worked examples — Wisconsin's two biggest metros

Most people start in or near one of these two. Wisconsin has an ADRC in every county — find yours by where you live. DHS/DQA licensing and the IRIS certification process are statewide; the local piece is mainly your municipality and fire authority.

- **Milwaukee (Milwaukee County):** ADRC of Milwaukee County is your entry point (statewide line 844-WIS-ADRC); several Family Care MCOs serve Milwaukee County (see the Family Care GSR map), and IRIS is also available; DHS/DQA via the Provider Portal licenses a 3–4 bed, with the City of Milwaukee / Milwaukee County for local ordinances and fire. The quirk: it's the state's biggest market with multiple MCOs — confirm which MCO(s) have a need in your specific municipality.
- **Madison (Dane County):** ADRC of Dane County — 2865 N. Sherman Ave., Madison, WI 53704; (608) 240-7400; Family Care MCO(s) serving Dane County plus IRIS; DHS/DQA via the Provider Portal for a 3–4 bed, with the City of Madison / Dane County for local ordinances and fire. The quirk: Madison is the state capital, so DHS and IRIS management are headquartered here, which can make in-person help easier.

To find your own county's ADRC and offices: call 844-WIS-ADRC (844-947-2372) to reach your county's ADRC (there's one in all 72 counties); ask the ADRC and local MCO(s)/IRIS whether there's a need, then start certification (1–2 bed) or the DQA license (3–4 bed); and call your city, village, or town about local ordinances and zoning — Wisconsin requires it.

12.4 Core rules & references

- **Wis. Stat. ch. 50 & Wis. Admin. Code ch. DHS 88:** licensed adult family homes (3–4 bed).
- **Wisconsin Medicaid Standards for Certified 1–2 Bed Adult Family Homes, P-00638:** certification standards for the 1–2 bed home.
- **Wis. Admin. Code ch. DHS 83:** Community-Based Residential Facilities (CBRFs, 5+ residents).

- **Wis. Admin. Code ch. DHS 12 & Wis. Stat. § 50.065:** caregiver background checks.
- **Family Care and IRIS program rules (1915(c) waivers):** MCO contracts and IRIS self-directed budgets.

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I have to open a licensed 3–4 bed home, or can I start smaller?

You can start smaller. A certified 1–2 bed adult family home — where you live in the home and support one or two adults — is the fastest way in and the true family model, certified through your MCO (Family Care) or DHS (IRIS) rather than licensed. It's the best way to learn the system before you take on a licensed 3–4 bed home (Path B) or a CBRF. Many providers grow from a 1–2 bed home to a 3–4 bed home.

Why do I have to deal with an MCO or IRIS — isn't this just Medicaid?

Wisconsin runs its long-term care through managed care and self-direction, so the money doesn't come from plain fee-for-service Medicaid. Under Family Care the state pays an MCO that buys services and pays providers; under IRIS the participant manages a budget and pays providers through a fiscal employer agent. So beyond being certified or licensed by DHS, you contract with an MCO or serve IRIS participants — that's how you actually get paid.

Does the funding pay for the person's rent and food?

No — the Family Care or IRIS payment is for the CARE and services above the level of room and board, not the resident's rent and food. The resident pays their own room and board from their own income, usually SSI. Keep the two completely separate in your agreements and your books; mixing them is a common and serious mistake.

What's the difference between an adult family home and a CBRF?

Size and regulation. A 1–2 bed and a 3–4 bed adult family home are small private residences (the 1–2 bed is certified, the 3–4 bed is licensed under DHS 88). A CBRF serves 5 or more residents and is licensed under DHS 83 with substantially heavier staffing, training, fire, and building requirements. Most new providers start with an adult family home and grow into a CBRF only later.

You can do this

Opening an adult family home in Wisconsin is real work — the managed-care and self-direction systems add a couple of extra steps — but it's a real, walkable path, and people across the state do it every day, giving someone a real home in the community. You don't have to be a lawyer or hire a consultant. You need a safe home, a good heart, and a clear map. Whether you start with a certified 1–2 bed home or a licensed 3–4 bed home, take it one step at a time: set up the business, confirm the need, get certified or licensed, contract with an MCO or serve IRIS participants, ready the home, and build the relationships that bring you placements. Verify the current details with the State as you go, and lean on your ADRC and your MCO/IRIS — that's what they're there for.

***Reminder.** Prepared for PolicyReady.org and grounded in Wisconsin DHS rules, the adult family home statutes and codes (Wis. Stat. ch. 50, Wis. Admin. Code ch. DHS 88, and DHS 83 for CBRFs), and the Family Care and IRIS program standards as of 2026. This is general information, not legal, tax, or billing advice. Requirements, rates, and codes change — always verify current details with DHS, DQA, your ADRC, the MCOs, IRIS, your municipality, and qualified counsel before you act.*